

Stroke Coordinator

Comprehensive Stroke Center (CSC)

Aligned with the 2026 AHA/ASA Guideline for Early Management of Acute Ischemic Stroke and AHA Quality Initiatives

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Position Summary

Coordinates and oversees the hospital's Comprehensive Stroke Center (CSC) program, ensuring care delivery meets current American Heart Association/American Stroke Association (AHA/ASA) evidence-based benchmarks — including the 2026 guideline update — for acute ischemic stroke (AIS), hemorrhagic stroke, and endovascular therapy (EVT), and that the program performs to Target: Stroke and Get With The Guidelines–Stroke (GWTG-Stroke) standards. A full glossary of abbreviations used throughout this description appears at the end of the document.

Key Responsibilities

Program Management

- Maintain protocols aligned with the 2026 Guideline for the Early Management of Patients With AIS, which replaces the 2018 guideline and its 2019 update.
- Lead the transition of stroke protocols to reflect 2026 practice changes, including:
 - **Tenecteplase:** add as an approved thrombolytic option alongside alteplase within the 4.5-hour window, based on trial evidence showing noninferiority and possible advantages over alteplase.
 - **EVT eligibility:** update criteria to capture patients with basilar artery occlusion and large ischemic cores who were previously excluded from intervention.
 - **Extended-window thrombolysis:** build a pathway for the 4.5–9 hour window in select patients based on advanced imaging, and prioritize treatment for disabling deficits regardless of NIHSS score.
 - **EMS bypass protocols:** review against updated guidance to transport to the closest EVT-capable hospital when rapid interhospital transfer systems aren't well established.
 - **Mobile stroke units:** evaluate feasibility of integration, now endorsed in the 2026 guideline, if applicable to the region's stroke system.
 - **Pediatric AIS pathway:** establish a pathway addressing the guideline's first-ever pediatric stroke recommendations.
 - **Hyperglycemia and dysphagia management:** update protocols per new evidence incorporated in the 2026 update.

Clinical Coordination — AHA/ASA Time Targets

- **Door-to-needle (DTN):** within 60 minutes in 85% or more of patients treated with IV thrombolytics, with secondary goals of 45 minutes in 75% of patients and 30 minutes in 50%.
- **Door-to-device (thrombectomy):** within 90 minutes for direct-arriving patients.
- **Door-to-groin puncture:** commonly tracked at 90 minutes or less.
- **Door-in-door-out (DIDO) for transfers:** ≤90 minutes in 50% or more of transferred patients.
- Implement AHA best-practice bundles: EMS prenotification, single-call stroke team activation, rapid imaging acquisition/interpretation, and standardized protocols.
- Screen incoming and in-house patients using updated large-core and basilar occlusion EVT criteria; ensure imaging protocols support extended-window thrombolysis decision-making.

Data & Quality Improvement

- Abstract and submit data to GWTG-Stroke; track tenecteplase vs. alteplase utilization and outcomes as the protocol transitions.
- Monitor EVT volumes and outcomes for the newly eligible large-core and basilar occlusion populations, since these represent an expanded case mix.
- Present GWTG-Stroke and Target: Stroke performance data, plus 2026-guideline implementation progress, to the stroke committee and hospital leadership.

Education & Outreach

- Retrain ED, ICU, neurology, and neurointerventional staff on 2026 guideline changes — particularly the new thrombolytic option, expanded EVT criteria, and extended treatment windows.
- Update EMS partner training on triage/transport protocols reflecting the "closest EVT-capable hospital" transfer guidance.
- If pursuing pediatric stroke capability, coordinate education for ED and pediatric staff on the new pathway.

Regulatory/Survey Readiness

- Maintain documentation showing protocols, order sets, and competencies have been updated to reflect the 2026 guideline (surveyors will expect current-guideline alignment).
- Track and document sustained achievement of AHA time targets under the revised protocols.

Qualifications

- **Education:** BSN required; MSN often preferred.
- **Experience:** 5+ years in neuroscience/neuro ICU or high-acuity stroke care.
- **Certifications:** SCRN strongly preferred, NIHSS certification, ACLS.
- **Skills:** GWTG-Stroke registry/data abstraction, familiarity with AHA quality-improvement toolkits, experience leading protocol/order-set revisions tied to guideline updates.

Glossary of Abbreviations

- **AHA** — American Heart Association
- **ASA** — American Stroke Association
- **CSC** — Comprehensive Stroke Center
- **PSC** — Primary Stroke Center
- **AIS** — Acute Ischemic Stroke
- **EVT** — Endovascular Therapy (mechanical thrombectomy)
- **IV tPA** — Intravenous Tissue Plasminogen Activator (a clot-dissolving thrombolytic medication)
- **DTN** — Door-to-Needle time (arrival to start of IV thrombolytic administration)
- **DIDO** — Door-in-Door-out time (arrival to departure for patients being transferred to a higher level of care)
- **NIHSS** — National Institutes of Health Stroke Scale (a standardized tool for scoring stroke severity)
- **GTWG-Stroke** — Get With The Guidelines–Stroke (AHA's national stroke data registry and quality program)
- **SCRN** — Stroke Certified Registered Nurse
- **CNRN** — Certified Neuroscience Registered Nurse
- **ACLS** — Advanced Cardiovascular Life Support (certification)
- **BSN** — Bachelor of Science in Nursing
- **MSN** — Master of Science in Nursing
- **RN** — Registered Nurse
- **ED** — Emergency Department
- **ICU** — Intensive Care Unit
- **EMS** — Emergency Medical Services
- **PI** — Performance Improvement
- **mRS** — modified Rankin Scale (a scale measuring degree of disability/dependence after stroke)
- **LVO** — Large Vessel Occlusion
- **SAH** — Subarachnoid Hemorrhage